

Hormonal therapy for womb cancer

Hormonal therapies can slow down or prevent cancer cells from growing. They can be used to treat womb cancer.

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What is hormonal therapy?

Hormones are substances produced naturally in the body. They act as chemical messengers and affect the growth and activity of cells. Oestrogen and progesterone are hormones that affect the growth of cells in the [lining of the womb](#). Hormonal therapies are drugs that change the way hormones are made or how they work in the body.

When is hormonal therapy used for womb cancer?

Cancer doctors may suggest hormonal therapy for [womb cancer](#) when:

- it is [advanced](#) (stage 4)
- it has come back after treatment (recurrent womb cancer)
- [surgery](#) or [radiotherapy](#) is not suitable because of other health problems.

The aim of hormonal therapy is to help shrink the cancer and control symptoms.

Hormonal therapy for advanced or recurrent womb cancer

The main hormonal therapy for womb cancer that is advanced or come back is progesterone. You usually have it as tablets. The most common types of progesterone are:

- [medroxyprogesterone acetate](#) (Provera®)
- [megestrol](#) (Megace®).

Other hormonal therapies are sometimes used, such as [letrozole](#) (Femara®).

Hormonal treatment for low risk early stage womb cancer

Cancer doctors may also suggest hormonal therapy instead of surgery if you would like to get pregnant in the future (fertility-sparing treatment). It is only suitable if you have [early stage and low grade](#) endometrioid cancer.

Fertility-sparing treatment

[Surgery](#) to remove the womb and ovaries is usually the main treatment for womb cancer. But having this surgery means you can no longer get pregnant. Sometimes it is possible to have fertility-sparing treatment instead if you would like to get pregnant in the future. It can only be used if you have:

- not been through the menopause
- [early stage and low grade](#) endometrioid cancer.

Fertility-sparing treatment usually involves taking daily progesterone tablets. Sometimes, progesterone is given directly into the womb through a hormone-releasing intrauterine device. This is sometimes called an IUD or coil.

You will have regular checks during and after fertility-sparing treatment. This is because there is a risk that the cancer may not respond to treatment, or it may come back.

You will need a total hysterectomy and bilateral salpingo-oophorectomy (BSO):

- if the cancer does not respond to hormone treatment, or it comes back again
- after you have children.

Your cancer doctor can explain the possible risks and benefits of fertility-sparing treatment.

Side effects of progesterone

The most common side effects of progesterone are:

- increased appetite
- weight gain
- headaches
- sleep problems (insomnia)
- constipation
- feeling sick
- feeling tired
- fluid build-up – such as swollen ankles (oedema).

Side effects can be mild and you are not likely to get all of them. Some will improve after the first few weeks of treatment. Some will get better after treatment finishes. Tell your doctor or nurse if you notice any side effects. They can often be managed.

Having cancer, and cancer treatments such as hormonal therapy, can increase the risk of a [blood clot](#).

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our womb cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.



Womb cancer forum

An anonymous network of people affected by womb cancer which is free to join. Share experiences, ask questions and talk to people who understand.

[Get help](#)